

Founder Q&A.

Pre-approved answers to the questions journalists most commonly ask about Mino Peptides, written in the voice of the founders and ready to be quoted directly or paraphrased in coverage.

This document provides PR partners with pre-approved responses to the questions Mino founders are most frequently asked. The answers are written in the founders' voice and are approved for direct quotation, paraphrase, or use in pitching conversations with journalists.

The document is organized into four sections: questions about the company and its mission, questions about the portfolio and sourcing, questions about the regulatory environment, and questions about the founders themselves. PR partners should familiarize themselves with all four sections before any media conversation about Mino.

For questions not covered here, route to Mino directly through hello@mino.life with the subject line "Q&A Request." New questions and answers will be added to future editions of this document as patterns emerge.

The company and its mission.

Q. What is Mino Peptides, in plain language?

Mino is a wholesale supplier of pharmaceutical-grade peptide therapeutics to licensed clinical practices. We do not sell to consumers, and we do not operate clinics ourselves. Our customers are credentialed medical practitioners who incorporate peptide compounds into patient care, and our job is to give them a supply chain they can stand behind.

APPROVED FOR ATTRIBUTION TO ELIZABETH STRAUS

Q. Why did you start Mino?

For years, the peptide category was served by a patchwork of research chemical vendors, repackagers, and unverified international sources. The clinicians doing serious work in regenerative and longevity medicine were forced to choose between expedience and integrity, and too often the patient paid the cost of that compromise. We built Mino because we thought medical-grade therapeutics deserved the same sourcing and documentation standards as the rest of a modern clinical practice.

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Q. How is Mino different from other peptide suppliers?

Four things. First, we work directly with cGMP-compliant manufacturing partners rather than through grey-market intermediaries. Second, we verify every lot through independent third-party analytical testing and ship the Certificate of Analysis with the order. Third, we label our compounds honestly. FDA-approved, investigational, and research-only compounds are not the same thing, and our documentation reflects that. Fourth, we only sell to credentialed clinical practices. We are not a consumer brand and we do not pretend to be.

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Q. What does “the quiet infrastructure of modern medicine” mean?

It means that the best supply chain is the one a clinician never has to think about. When you walk into a hospital pharmacy, you do not interrogate where the heparin came from or whether the vial was stored correctly. You trust the infrastructure. We want Mino to earn that same kind of quiet trust from the practitioners who use the compounds in our portfolio.

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The portfolio and sourcing.

Q. How many compounds does Mino offer, and what do they cover?

Our portfolio spans thirty-nine compounds across six therapeutic categories. The categories are Regeneration, Metabolic, Longevity, Performance, Cognitive, and Vitality. Each compound is selected because a credentialed clinician might reasonably incorporate it into patient care within an appropriate regulatory framework. The portfolio is deliberately focused rather than exhaustive.

APPROVED FOR ATTRIBUTION TO BRANDON HOWARD, MD

Q. Where do your compounds come from?

We work directly with cGMP-compliant manufacturing partners. These are facilities operating under current good manufacturing practice standards, which is the same regulatory framework that governs pharmaceutical manufacturing more broadly. We do not source through intermediaries or repackage research chemicals from unverified upstream suppliers, and we maintain documented chain of custody for every lot.

APPROVED FOR ATTRIBUTION TO BRANDON HOWARD, MD

Q. How do you verify quality?

Every lot we receive is verified through independent third-party analytical testing, and a Certificate of Analysis ships with the compound to the clinical account. The COA confirms identity, purity, and endotoxin levels to pharmaceutical specification. Storage and handling protocols ship with every order, because compound stability matters and a verified product handled poorly is not a verified product anymore.

APPROVED FOR ATTRIBUTION TO BRANDON HOWARD, MD

Q. What about cold chain logistics?

Cold chain is non-negotiable for the compounds in our portfolio that require it. We ship through a temperature-monitored network with tracked delivery and signature-required receipt. Enterprise accounts can request overnight and same-day options. The supply chain we built was designed for pharmaceuticals, not parcels.

APPROVED FOR ATTRIBUTION TO ELIZABETH STRAUS

The regulatory environment.

Q. The peptide category has had a complicated regulatory history. How do you navigate that?

Honesty about the regulatory landscape is one of our operating principles, and we think it is actually a competitive advantage rather than a burden. Every compound in our catalog is labeled with its current status: FDA-approved as a branded agent, investigational, research-only, or cosmetic. Those are not interchangeable categories, and we do not let them blur in our sales conversations or our documentation. The clinicians who buy from us appreciate that, because they have to operate within those distinctions every day.

APPROVED FOR ATTRIBUTION TO BRANDON HOWARD, MD

Q. What is your view on recent FDA enforcement around compounding pharmacies?

The compounding landscape has shifted considerably over the past year, particularly around GLP-1 and related compounds. Our position has been to work only with 503A and 503B facilities whose operating standards meet our expectations and who have not been the subject of adverse enforcement action. We watch the enforcement environment closely and we update our partner relationships when we need to.

APPROVED FOR ATTRIBUTION TO ELIZABETH STRAUS

Q. How do you handle the distinction between off-label prescribing and research-use compounds?

That distinction is critical and we treat it that way. An approved drug prescribed off-label is a different legal and clinical category from a research-only compound dispensed in an investigational framework. We label, package, and document every shipment in a way that reflects which is which, and we expect our accounts to maintain the same discipline. The credentialing process exists in part to confirm that they will.

APPROVED FOR ATTRIBUTION TO BRANDON HOWARD, MD

Q. Are you worried about the regulatory environment becoming more restrictive?

We think the regulatory environment is going to keep evolving, and we have built the company to evolve with it. Companies that depend on regulatory ambiguity for their business model will struggle as the rules clarify. Companies that have already built to a higher standard will benefit. We made our choice on which side of that line we wanted to be on when we started Mino.

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The founders.

Q. What is your background, and how did it lead you to peptides?

I spent most of my career in healthcare operations. I was Executive Vice President at CareOne, where I helped oversee a network of more than eighty facilities, and I co-founded 18 Medical MSO, which became one of the largest independent physician management organizations in New Jersey. That background gave me a clear view of where supply chain integrity matters in clinical practice and where the existing peptide infrastructure was failing the practitioners who needed it most. Mino is what I built in response.

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Q. Dr. Howard, what drew you to this work?

I came to peptide therapeutics through internal medicine. The clinical principles I was taught in residency, that practice is the practice of evidence and judgment and continuity of care, do not become optional when the modality shifts. They become more important, because the clinician is often working at the frontier of the literature. I wanted to be part of building an infrastructure where regenerative and longevity medicine could be practiced with the same rigor as any other clinical specialty, and Mino and RegenRX together are how we are doing that.

APPROVED FOR ATTRIBUTION TO BRANDON HOWARD, MD

Q. What is your relationship with RegenRX Health Institute?

RegenRX Health Institute is the clinical platform side of what we are building. It is anchored by a flagship facility in Paramus, New Jersey, and it extends through a growing network of physician partner locations. Dr. Howard is the Medical Director. The two businesses share founding leadership and operating philosophy, but they serve different functions. Mino supplies the compounds. RegenRX practices the medicine.

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Q. What does success look like for Mino five years from now?

Success is when a credentialed practitioner anywhere in the country can order a peptide from Mino with the same confidence they would order any other pharmaceutical, and when the broader category has caught up to the standards we set out to build to. We are not trying to be the biggest peptide supplier in the market. We are trying to be the one the most serious clinicians choose, and the one that helps the category mature.

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